

Sacral Neuromodulation (Bladder and Bowel Pacemaker)

WARWIKI

Patient Information · An implanted device that steadies bladder and bowel control

Sacral neuromodulation (SNM) uses a small implanted device, sometimes called a “bladder pacemaker.” It gently stimulates the **sacral nerves** near the tailbone, which help control the bladder and bowel. It is used for overactive bladder, some types of trouble emptying the bladder, and accidental bowel leakage. A key strength is that you **test it first**, before the permanent implant.

How It Works

The sacral nerves carry signals between the bladder, the bowel and the brain. SNM sends **mild electrical pulses** that help steady those signals. This can calm urgency and leaks, or help a bladder that does not empty. You can adjust or pause it with a handheld remote.

What It Treats

- **Overactive bladder** (urgency, frequent trips, urge leaks)
- **Urinary retention** not caused by a blockage (a bladder that will not empty)
- **Accidental bowel leakage** (fecal incontinence)

Benefits and Risks in Numbers

about
7 in 10

with urge leakage were still helped 5 years after the implant

about
1 in 3

needed another procedure on the device within 3–4 years

about
4 in 100

got an infection at the implant site in the first year

Numbers are averages from published studies. Your own risk may differ; ask your team.

Learn the Terms

Sacral nerves

Nerves near the tailbone that help control the bladder and bowel.

Neuromodulation

Using gentle electrical pulses to steady nerve signals.

Test phase

A trial with a thin wire to see if it helps before the permanent implant.

Generator

The small battery device placed under the skin of the upper buttock.

Lead

The thin wire that carries the pulses to the nerve.

Remote

A handheld controller to adjust or pause the device.

MRI

A scan that uses a strong magnet. Some devices limit which MRI scans you can have.

How do I know if it will work for me? You **try it first**. A thin wire is placed and you track your symptoms in a diary for a few days to a few weeks. You move on to the permanent implant only if your symptoms improve by about half or more.

Test phase

Keep a diary. Avoid bending, twisting and hard activity so the wire does not move.

After the implant

Some soreness for a few days. Your team sets up the device.

Over the years

Setting changes, battery care, and sometimes another procedure.

How It's Done (Two Steps)

- 1 Test phase:** a thin wire is placed near a sacral nerve, in the office or an operating room. You get numbing medicine, sometimes with medicine to relax you. The wire connects to a small device worn outside the body.
- 2 Implant:** if the test works, a small **generator** is placed under the skin of the upper buttock and joined to the wire. You usually go home the same day.
- 3** Your team programs the device. You use a **remote** to adjust it.

Call the office that did your procedure if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Redness, swelling or fluid at the wound
- New or worse pain at the device site
- A sudden loss of benefit, or stimulation that feels uncomfortable

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Living With the Device

- Some batteries are recharged at home. Others last many years and are then replaced.
- **Before any MRI**, show your device card. Many newer systems allow MRI scans under set conditions. Some older wires allow head scans only.
- Never have **diathermy** (deep heat therapy).
- If you become pregnant, tell your team. The device is usually turned off.

Three Things to Remember

- 1. You test it first.** The permanent implant is placed only if it clearly helps.
- 2. It can be adjusted, turned off or removed.**
- 3. Show your device card before any MRI.**